

MEQ PRACTICE QUESTIONS (CASE STUDIES)

MODIFIED ESSAY QUESTION 12

26-year male with a history of Schizoaffective disorder and methamphetamine dependence.

The patient is released from prison 2 weeks ago on clopixol depot and olanzapine. Currently on enforced treatment MHA (not forensic licence).

He tells you that he is drug free as he is required to do random UDS for the next 6 months as part of his bail condition.

He came to see you with his sister who is adamant that he should not be on a treatment order, and that he has rights to choose his own treatment.

a) List and explain the possible ethical issues that arises from this situation.

Sister confirms that they will be relocating to rural area soon. He will be referred to the CMH in the region for follow up.

b) What are the possible risks involved in terms of historical, social and clinical aspects in this man with regards to relapse?

c) List and explain principles of informed consent.

MODIFIED ESSAY QUESTION 13

You are a junior consultant psychiatrist on call for the Emergency Department at a general hospital. Early on a Friday evening you are called for advice by a first-year psychiatry registrar on duty.

The registrar has reviewed Toby, a 28-year-old man who is new to the area. He had presented asking for a prescription of diazepam 5mg tablet, up to four tablets a day with five repeats. He says he has been taking these for a while for his “nerves” and he has run out. There is no information available on Toby in the hospital records system.

- a) Outline (list and justify) the key information on history you would expect the registrar to have obtained. Please note: a list with no justification will not receive any marks. (12 marks)**

The registrar telephones you two weeks later to inform you that Toby did not attend a follow-up appointment at the outpatient department. The registrar also informs you that on the same day he had received an email from the hospital asking him to respond to a complaint lodged by Toby about “the poor care” Toby had received during the ED consultation.

- b) Describe (list and explain) what you would discuss with the registrar about managing this situation further. Please note: a list with no explanation will not receive any marks. (9 marks)**

MODIFIED ESSAY QUESTION 14

12-year-old Aboriginal boy brought to the emergency department by the police after damaging property and threatening to hurt himself.

On arrival to the ED he was settled and was playing cards with his carer. He recently moved to his 10th group home. His care was transferred to the State when he was 5 after witnessing family violence and his father killed his mother and was incarcerated.

a) List and explain possible explanation of his behaviour.

He is treated with olanzapine, sertraline, long-acting Methylphenidate and sodium valproate. Carer informs he is treated for childhood bipolar disorder.

b) Discuss and debate possible differential diagnosis.

Patient is under a therapist and has a behavioural management plan in place- he is rewarded when 'good' When behaving badly privilege is taken away. However, this is not working.

c) Carer asks why is the behaviour management not working and how else can they manage?